Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

Op. Dr. Murat Ayata Private Clinic
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INFORMED CONSENT FORM

Tympanoplasty

Repair of the eardrum (with middle-ear bone repair and mastoidectomy if needed)

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Side: ☐ Right ☐ Left

Visualisation: ☐ Microscopic ☐ Endoscopic ☐ To be decided during surgery

_____

Approach: ☐ Transcanal ☐ Postauricular ☐ Endaural _____

Graft source: ☐ Muscle sheath (temporalis fascia) ☐ Cartilage / perichondrium ☐ Other _____

Ossicular repair (ossiculoplasty): ☐ Not planned ☐ Planned ☐ If needed _____

Mastoidectomy: ☐ Not planned ☐ Planned ☐ If needed _____

Date and summary of pre-operative hearing test (audiogram): _____

1 Your condition and the aim of the procedure

The eardrum is a thin membrane that separates the ear canal from the middle ear and passes sound vibrations to the tiny bones (ossicles) in the middle ear. When there is a **hole (perforation)** in the eardrum, hearing is reduced and water or germs entering the ear cause **recurrent ear discharge**.

Eardrum perforations are mostly caused by **chronic middle-ear infection**, and sometimes by injury (a slap, a cotton bud, a sudden pressure change). Small, recent holes may heal on their own; however, long-standing holes usually do not close. In some patients the infection may also have worn away the middle-ear bones or spread into the air cells in the bone behind the ear (mastoid). If there is a **cholesteatoma** in the middle ear (a sac of skin cells that grows and erodes bone), its treatment is particularly important.

The aim of tympanoplasty is to close the eardrum, clear the middle ear of infection to achieve a **dry and safe ear**, and, where possible, to **preserve or improve hearing**.

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2 How is the procedure performed?

The operation is performed under general anaesthesia in an operating theatre, using a microscope and/or an endoscope.

1. **Approach:** If the hole is small and well placed, the operation can be done entirely through the ear canal, leaving no visible scar. For larger holes or when a wider view is needed, a small incision is made behind the ear (in the crease behind the ear) or in front of the ear.
2. **Taking the graft:** The patient's own tissue is used to repair the eardrum: usually the sheath of the muscle above the ear (temporalis fascia) or the cartilage and its lining from the ear. These tissues are taken through the same incision or a small additional one.
3. **Clearing the middle ear:** The edges of the hole are freshened, the eardrum is lifted and the middle ear is inspected; any infected tissue and adhesions are cleared. The ossicles are checked.
4. **Ossicular repair (if needed):** If the chain of ossicles is eroded or broken, it may be reconstructed using the patient's own bone/cartilage or a tissue-friendly prosthesis such as titanium.
5. **Mastoidectomy (if needed):** If infection or cholesteatoma has spread into the air cells of the bone behind the ear, these cells are opened and cleared with a special drill.
6. **Placing the graft and closure:** The graft is placed under or over the hole. The middle ear and ear canal are supported with dissolvable sponges. Any incision is closed with stitches and a pressure dressing is applied over the ear.

If cholesteatoma is found, in some patients a **second-look operation 6–12 months later** may be planned to make sure the disease has been completely removed. This will be explained to you separately.

ANAESTHESIA	APPROXIMATE DURATION	HOSPITAL STAY / DISCHARGE
General anaesthesia	About 1.5–3 hours (longer with mastoidectomy)	Usually same day or 1 night

3 Expected benefits and likelihood of success

- Closure of the hole in the eardrum
- Fewer or no more episodes of ear discharge and infection; a dry and safe ear
- Fewer restrictions needed to protect the ear when bathing, showering and swimming
- Preservation or improvement of hearing in suitable patients
- If cholesteatoma is present, removal of the disease and prevention of its possible serious complications

Permanent closure of the eardrum is achieved in about **85–95%** of patients. Improvement in hearing depends on the condition of the ossicles, the extent of infection and the health of the inner ear; hearing may improve significantly, stay the same or, rarely, get worse. If the graft does not take or a new hole forms, the operation can be repeated.

4 Other treatment options

- **Observation and keeping the ear dry:** For small, dry holes, regular check-ups and protecting the ear from water may be an option. The hole stays open and the risk of infection continues.
- **Medical treatment:** Antibiotic ear drops control infection temporarily during episodes of discharge; they do not close the hole.
- **Hearing aid:** Can compensate for hearing loss; however, it may be difficult to use in a discharging ear and does not close the hole.
- **Simple patching for small holes:** For very small holes, closure with a paper patch or fat (myringoplasty) may be considered.

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5 What happens if the procedure is not performed?

Without surgery, the hole in the eardrum usually stays open. Discharge may recur when water gets into the ear or with colds, and hearing loss may increase over time. If cholesteatoma is present and left untreated, it continues to grow and can lead to serious complications such as **permanent hearing loss, dizziness, facial paralysis, meningitis or brain abscess**.

You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

6 Preparing for the procedure

- A hearing test (audiogram) is done before surgery and, if necessary, a CT scan. You will be assessed by the anaesthetist.
- You must not eat or drink anything for **6–8 hours** before the operation (please follow the anaesthetist's instructions).
- If the ear is actively discharging, you may be asked to use drops before surgery to dry the ear.
- On the day of surgery, a little hair behind the ear may be trimmed. If you have flu or a respiratory infection, surgery may be postponed.
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.
- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

7 Recovery: what to expect

First 24–48 hours	You will have a feeling of fullness, mild pain and blockage in the ear; the pain is controlled with painkillers. Brief mild dizziness and nausea may occur. The pressure dressing is usually removed on day 1. Avoid lying on the operated ear.
Weeks 1–2	Because of the sponges in the ear canal, hearing will feel **reduced** during this period; this is normal. There may be slight blood-stained discharge from the ear. Stitches are usually removed on **days 7–10**. Most people return to desk work within 1 week.
Weeks 3–6	The sponges in the ear canal are cleaned out at check-ups or dissolve on their own. Water must not get into the ear. Strenuous exercise, heavy lifting and straining should be postponed for about **4–6 weeks**. Air travel is usually possible **after 2–4 weeks**, with your doctor's approval.
Months 2–3	Whether the graft has taken is assessed at check-ups. A hearing test at this stage shows the effect of the operation on hearing. Swimming and water exposure are usually allowed after your doctor's approval.
Months 6–12	The eardrum and middle ear complete their healing. After cholesteatoma surgery, regular check-ups and, if necessary, imaging or a second-look operation are planned.

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8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

Expected, temporary effects (not complications — a normal part of healing)

- A feeling of fullness and blockage in the ear, and hearing feeling reduced in the first few weeks (due to the sponges in the ear canal)
- Mild ear pain, crackling, sloshing or buzzing sounds
- Slight blood-stained discharge from the ear (first few days)
- Temporary numbness of the ear and around the incision; the ear sticking out slightly (after the behind-the-ear approach; this settles within weeks)

Common conditions (most are temporary or resolve with treatment)

- **Change in taste** — The taste nerve (chorda tympani) runs just behind the eardrum and may be stretched during surgery or, rarely, have to be cut. About **5–20 in 100** patients notice a metallic taste, reduced taste or a dry feeling on that side of the tongue; this usually **improves or becomes unnoticeable within weeks to months**. Rarely it may be permanent.
- **Tinnitus (ringing in the ear)** — Ringing in the ear may occur after surgery, or existing ringing may increase for a while; it is usually temporary.
- **Temporary dizziness** — Mild unsteadiness or dizziness may occur in the first few days; it usually settles within a few days.

Uncommon conditions

- **Graft failure or a new hole** — In about **5–15 in 100** patients the eardrum may not close completely or a hole may form again later. In this case discharge may continue and the **operation may need to be repeated**.
- **Hearing does not improve** — Even if the eardrum closes, hearing may not improve as expected because of the ossicles, middle-ear ventilation or the inner ear. After ossicular repair, the prosthesis may move or be pushed out through the eardrum; a further operation may be needed.
- **Ear infection and discharge** — Ear discharge or wound infection may occur after surgery; it usually settles with drops or antibiotics. Infection of the ear cartilage (perichondritis) is rare but can leave a deformity of the cartilage.
- **Narrowing of the ear canal or lateralisation of the eardrum** — Scar tissue may narrow the ear canal, or the new eardrum may move away from its correct position; this may affect hearing and require a corrective procedure.
- **Recurrent or residual cholesteatoma** — After cholesteatoma surgery, the disease may recur or a small residue may grow in some patients; long-term follow-up and, if necessary, **further surgery** are therefore required.
- **Wound and scar problems** — A thickened scar or keloid at the behind-the-ear incision, and rarely a fluid collection behind the ear, may occur.

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Rare or very rare, but potentially serious conditions

- **Permanent hearing loss due to inner-ear damage** — In **fewer than 1 in 100** patients, **permanent sensorineural hearing loss** may develop because the inner ear has been affected; very rarely hearing in the operated ear may be lost completely (dead ear). This may require a hearing aid or further hearing rehabilitation. The risk is higher with cholesteatoma and disease close to the inner ear.
- **Prolonged dizziness and imbalance** — Rarely, the inner ear may be affected, causing dizziness that lasts for weeks; balance exercises and medication may be needed.
- **Facial nerve paralysis** — Because the facial nerve passes through the middle ear, very rarely (**far fewer than 1 in 100** patients) temporary or permanent weakness may develop on that side of the face. The risk is higher with cholesteatoma, repeat operations and nerves with an unusual course from birth. If it occurs, medication, eye protection and, if necessary, surgery (decompression or repair of the nerve) may be needed. If it is permanent, facial asymmetry and incomplete eye closure may remain, requiring long-term eye protection and possibly facial reanimation surgery.
- **Complications involving the brain lining** — Especially in mastoidectomy and cholesteatoma surgery where the bone of the skull base has become thin, very rarely cerebrospinal fluid (CSF) leak, meningitis or injury to the brain lining may occur. This may require further treatment in hospital (sometimes in intensive care) and surgery; meningitis can rarely cause lasting harm.
- **Permanent change in the shape of the ear** — As a result of the behind-the-ear approach or the taking of a cartilage graft, a slight change in the position or shape of the ear may rarely be permanent.

General risks that apply to any surgical / medical procedure

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.
- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.
- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

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9 When to contact us immediately

- **Sudden, marked hearing loss** (a sudden worsening, different from the blockage caused by the sponges)
- **Severe dizziness**, vomiting and being unable to stand
- **Facial weakness**: drooping at the corner of the mouth, inability to close the eye
- **Fever above 38.5 °C**, increasing ear pain, redness and swelling of the ear, or foul-smelling, pus-like discharge
- **Clear, watery discharge** from the ear, severe headache or neck stiffness
- **Bleeding that does not stop** from the dressing or the ear

If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).

10 Medication and everyday-life advice

- Until your doctor allows it, **do not let any water into your ear**: when showering, plug the ear with cotton wool coated in petroleum jelly; do not go into a pool or the sea.
- **Do not blow your nose** for the first **4–6 weeks**; if you need to sneeze, sneeze with your mouth open. Do not pinch your nose and strain (do not perform the Valsalva manoeuvre).
- Avoid heavy lifting, straining, bending and strenuous exercise in the first few weeks. Tell your doctor if you are constipated.
- Always get your doctor's approval for air travel, diving and activities involving pressure changes; **diving** usually requires a longer wait.
- Do not put cotton buds or any other object into the ear canal; use the ear drops given to you as shown.
- If you have travelled from abroad, plan your return journey with your doctor; check-ups and hearing tests after your return can continue with a local ENT doctor.
- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.
- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.
- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

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12 Photographs, image records and your personal data

Photographs/videos may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

- ☐ I agree that my images are used **only for medical records and treatment follow-up**.
- ☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).
- ☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.
- ☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

13 Information and notes specific to you (completed by the doctor)

Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write "none".

14 Patient declaration

- ☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.
- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.
- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.
- ☐ I understand that the main aim of the operation is to close the eardrum and achieve a dry, safe ear; that improvement in hearing cannot be guaranteed and that, rarely, hearing may get worse.

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☐ It has been explained to me that, depending on what is found during surgery, ossicular repair or mastoidectomy may be needed, and I consent to these procedures being performed as marked above.

☐ **I GIVE MY CONSENT to the procedure described above.**

☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."

15 Signatures

	FULL NAME	SIGNATURE	DATE / TIME
Patient			
Legal representative (parent / guardian) — relationship:			
Doctor providing information and performing the procedure			
Interpreter (if needed) — language:			
Witness (if needed)			

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.

☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.